User Research Evidence

Interview transcripts, field notes and research summary

Capstone: Acko first-time health insurance buyers · **Researcher:** Sagar · **Collected:** November 2026

What this document contains

Part	What it is	People	How it was captured
Transcripts 1 to 7	Recorded user interviews: Adhira, Aditya, Manasa, Bhargav, Rekha, Saurabh, Abhinav	7	Phone or voice recordings (1.5 to 18.5 min), machine-transcribed and cleaned
Transcript 8	Field notes from informal conversations at a Diwali gathering, 8 Nov 2026	About 10	Not recorded; notes written from memory afterwards
Research summary	One combined summary of all 8 transcripts	17 in total	Merged from the three summaries written during the research, with repeated content removed

Each transcript is reproduced exactly as it was written; nothing in a transcript has been changed or shortened. Only the summaries were combined: the interview summary, the combined research summary and the field-notes synthesis covered the same themes, so their content appears once, in the research summary at the end.

Transcription key: [unclear] = speech couldn't be made out. [?] = best guess at a word.

Transcript 1: Adhira (recorded interview)

Recording: Adhira_UI.mp3 (3 min 11 s) · Interviewer: Sagar

[00:00] Sagar: Okay. So, you have insurance?

Adhira: Yes.

Sagar: I mean a personal insurance, apart from office?

Adhira: No.

Sagar: Okay. Why haven't you gotten yourself a personal insurance apart from your company coverage?

Adhira: Because I've been looking out and trying to research on what the best insurance would be, so I'm looking out.

[00:27] Sagar: Okay. Yeah. Okay. So [unclear] research. So you're doing the research currently?

Adhira: Yeah.

Sagar: Okay. So, while doing the research, you basically have to, you know, search for those insurances and see their features and everything, right? So that is basically you going and trying to purchase something.

Adhira: Mm-hmm.

Sagar: Right. So tell me the moment when you tried to look at the insurance and purchase it — you know, what made you not purchase the insurance?

Adhira: I don't know the answer, Sagar.

[01:13] Sagar: Yeah, so like, what made you not buy the insurance? Hello?

Adhira: I'm looking at the coverage and everything.

[01:33] Sagar: Hello. Hello. Are you able to hear me?

Adhira: Yeah, I'm sort of able to hear you.

Sagar: I'm asking you, like, what made you not [unclear — "go ahead with"?] the purchase?

Adhira: Not happy with the coverage.

[01:53] Sagar: So are you aware of the waiting period, sub-clauses and everything, or [unclear]?

Adhira: Yeah, the sub-clauses also.

Sagar: Okay, but are you aware of the waiting periods of specific diseases under insurance?

Adhira: No, I'm not aware.

[02:13] Sagar: What confuses you the most in insurance?

Adhira: There might be some clauses, and there might be certain insurance providing certain sort of coverage. I'm not really able to come to a final decision on what is the best offer provided by various agents.

Sagar: Okay. Have your parents asked you something about insurance and you were not able to answer?

Adhira: Yeah, multiple things, because I don't have [unclear — "that much"?] knowledge about it.

Sagar: Okay. Okay. Are you aware of this company called Acko?

Adhira: I've heard about it.

Sagar: [unclear] You don't know what they do?

Adhira: Yeah, yeah.

Sagar: Understood, understood.

Transcript 2: Aditya (recorded interview)

Recording: Aditya_UI.mpeg (1 min 27 s) · Interviewer: Sagar

[00:01] Sagar: Do you have a personal health insurance?

Aditya: No, I don't.

Sagar: Why haven't you thought of buying it yet?

Aditya: I never thought about it.

[00:17] Sagar: Okay, you have office health insurance?

Aditya: Yes.

Sagar: Do you think that is enough?

Aditya: Yes.

Sagar: What is the coverage you have?

Aditya: I don't remember right now.

[00:33] Sagar: Have you ever browsed through buying health insurance online?

Aditya: No, I have not.

[00:42] Sagar: Uh… are you aware about the waiting period clauses, sub-clauses of health insurance?

Aditya: No, I do not know.

Sagar: Okay. Has anything confused you about health insurance?

Aditya: No, I did not browse about it, so no.

[01:00] Sagar: Have your parents asked you anything about health insurance?

Aditya: No.

Sagar: Are you aware about this company Acko?

Aditya: I think I [unclear — "have heard"?] the name.

Sagar: Are you aware that they have health insurance?

Aditya: No [unclear].

Transcript 3: Manasa (recorded interview)

Recording: Manasa_UI.mpeg (11 min 28 s) · Interviewer: Sagar

[00:02] Sagar: The recording is [unclear — "started"?]. First question. Do you have a personal health insurance?

Manasa: No.

Sagar: You have an office one, correct?

Manasa: Yes, yes.

Sagar: Any reason you are not having a personal health insurance?

Manasa: Uh… the office gives a good cover, so I've always thought that's enough, but uh… I'm planning to.

[00:30] Sagar: What do you think is a good cover?

Manasa: Hmm. Maybe a… 10 to 15 lakhs cover for you, and then another 15 for a family, probably.

Sagar: Okay. Understood. [unclear]

[00:52] Sagar: Have you ever, you know, looked at buying a personal cover for yourself?

Manasa: I did look at a few options.

Sagar: Okay. Okay, now tell me the moment that, you know, you have gone online and you have checked for some covers, right? So what made you not buy any of them, and what would have made you, you know, purchase it? What were the difficulties that you found in the journey?

Manasa: So when I initially looked at a few… uh… since I already had an underlying condition of thyroid, there was a lot of extra premiums, or I would have a waiting period, or some would, you know, say this much weight of this person is also a problem, risk factors, or they would add extra premiums to it, or they would say I have to go through this much of waiting [period], only then it will be active. And for very less cover I had seen a high premium sometimes. [unclear] Yeah, those are all the reasons why I got back and then had to think twice about it.

[02:17] Sagar: Okay, so you're saying if you had some information on that even before paying for it and [unclear] or even before the rejection, right, you could be more clear?

Manasa: Yes, yes.

[02:35] Sagar: What was something that actually confused you and you had to check for help elsewhere?

Manasa: [unclear] I don't understand this whole thing of what's covered, what's not. Like, [unclear], what I've seen in some insurances, in the entire hospital bill there are certain sections of that bill that will not be covered by your insurance. They say, "We will not pay for these, we will pay only for this." And let's say there's a slab on your rooms — this kind of rooms. All of it — before you start an insurance, it should have the information [unclear], and we should know which they will agree to, which they will not agree to. Because when I go into a hospital, I'm thinking, "Oh, I have insurance." But then when I have gone — I've had experiences with my parents, [they] had to go in, and then I've seen — I assume they'll give me at least 80% of the bill, but I've got like 40–50%, so the other 50% I'll have to make up on my own then. So that kind of confusions should be cleared, I think, upfront when we take insurance, maybe.

Sagar: Understood. Okay.

[03:50] Sagar: So tell me something that your family asked you about health insurance and you couldn't answer.

Manasa: Uh… probably this — exactly what I just told you about. How do you know how much of the bill will I be able to pay through the insurance?

[04:14] Sagar: Are you aware about the waiting periods, the exclusions?

Manasa: [What's] covered, what's not covered — see, what's covered, what's not covered, there's like a huge list that—

Sagar: Exactly, [that] is there in the document. But at least about the basics, like, you know, what is covered during the waiting period and what [is not]? You're aware of that?

Manasa: No, see, that is what I'm saying. When you see whatever the terms and conditions are there — like you said, it's in detail, there's like 50 pages of terms and conditions. Nobody will have the time to sit and go through all the 50 pages of terms and conditions — "Oh, will this be allowed? Will this not be allowed?" and all that, right? So at least give — if there's a brief saying "this category of things or this category of things" — so it'll make me understand better.

[05:04] Sagar: Okay, so let me phrase it a different way. So let's say you have taken a health insurance which says there's a two-year waiting period for a cataract [surgery] to be done. And after buying the insurance, 10 months later, the person finds that the person requires a cataract surgery. What would you say? Who would pay the bill?

Manasa: If it's within that waiting period, then he's not going to — we have to pay the bill. After the waiting period, yes, the insurance pays the bill.

[05:39] Sagar: Are you aware of the add-on type of insurance offered, for you to cover all the exclusions as well? For example, you have a bill of 1 lakh rupees and they cover 70,000 rupees, and the rest they don't cover, citing that these are consumables, right — gloves, syringes and all these things. But you take an add-on by paying a few extra money and that is also covered.

Manasa: Oh, I didn't know about that. No, I didn't know.

Sagar: Okay.

[06:14] Sagar: Okay, what would you say would be your ideal premium that you would like to pay, and what is the ideal coverage that you would want to have?

Manasa: So… I would say a cover of maybe 15–20 lakhs, with the premiums [of] 15, 20,000 rupees. Then 15,000 rupees. This is what I hope to [pay]. A little higher than that was also fine if they're giving me a 15–20 [lakh cover].

Sagar: And you didn't find that budget anywhere else?

Manasa: Like I said, yeah, if I found it, I don't know the details of it. There are certain terms and conditions that I don't understand — all this waiting period and pre-existing — all those terms and conditions are there which I [don't] understand. So yeah, that is one of the problems why I am not able to find one.

[07:15] Sagar: Are you aware of Acko?

Manasa: [unclear] I have seen a lot [of ads?] about it, but never gone in and dived in to see anything about it.

Sagar: Okay. So when I was looking at Acko, they had a health insurance coverage of 25 lakhs for 8,000 rupees.

Manasa: Oh, okay.

Sagar: And they would pay 100% of your claim. There is no deductions — zero deductions. 100% claim settlement. This is what they offer, and that is their USP. Transparency is also what their USP is.

[08:05] Manasa: And is this like they cover most of the hospitals, or is it…?

Sagar: They have most of the hospitals covered. I mean, they're not like a traditional insurance like HDFC Ergo or something like that — you can get them from agents and everything, so there are a lot of commissions going. So this is D2C — direct to consumer. Okay, so they are selling the insurance directly to you. So no middleman is there, so they are able to offer low premiums. And it's recently started — I think they are 2 or 3 years old in the health insurance category. They were known for the vehicle insurance before.

[08:49] Manasa: Okay, what is their number? Like, how many insurances have they, in these two–three years, [settled] claims [for]?

Sagar: No, I don't know about that, but their claim settlement ratio is somewhere close to 97%.

Manasa: Okay.

Sagar: It's easy to say — it is equal to any of the other insurers' claims, right? But none of them [say], out of those 97%, what is the percentage of the claim that they have settled — for a 1 lakh claim, have they settled 100% of the claim or 80% of the claim? But these guys have a USP of 100% of any claim [unclear]. So, what do you think — is this something which is too good to be true, or is this something actually worth looking at?

[09:44] Manasa: It does sound like it's a little too good to be true, but I would want to go in detail, look at it, figure out myself if that is what I'm looking at. And I hope whatever they're claiming is true.

Sagar: Yeah, okay, let's see. In spite of that — you said, you know, they were charging you exorbitant amounts for the thyroid problem. Even if they charge you — I mean, obviously, because it is a pre-existing condition they will charge you more — but still it does not fall into that exorbitant amount, right? So if you have something like this on your table, is this something you will actually consider, or is it something that you would, you know, again evaluate with the traditional insurance options available?

[10:38] Manasa: I would have fairly considered, but like I said, I would also evaluate and do my market research. But I think I would definitely consider. Acko is one of my options.

[10:47] Sagar: Okay. If you're confused on any of the insurance things, to get clarity, where do you go?

Manasa: [unclear] And all these Policybazaar and all those places that are there out there, which give you [unclear — "unbiased"?] opinions of what is right, what is the right choice for you, maybe. Honestly, [I've] already used [them]. [unclear]

Sagar: [unclear] That's all my questions.

Transcript 4: Bhargav (recorded interview)

Recording: Bhargav_UI.mpeg (12 min 07 s) · Interviewer: Sagar

[00:02] Sagar: [unclear] …recording, bro? Yeah, it's done. So tell me something. This is with regards to the health insurance survey that I sent you, remember?

Bhargav: Yeah.

Sagar: It's regarding that. So, like, tell me, like, what happened, you know, that made you start looking for a health insurance?

[00:30] Bhargav: Technically, yeah, I've heard these stories from my parents there. You know, they have seen their friends or relatives having certain [unclear — "panic"?] issues or heart issues. You know, at that point of time, if you don't have any emergency savings or funding, that would be an issue. Because as you know, when you need treatments, having a health insurance would help you financially to save costs in some ways and also to get the required treatment. So that is why people educate us to get health insurance, because it gives you emergency savings — a certain amount of your salary is going in as a part of investments, so that would help you in those sorts of situations.

[01:18] Sagar: Now tell me the moment when you stopped, like when you were about to purchase [an insurance] online — the moment when you, you know, said "No, I'm not going to purchase," or you just made a decision of purchase. Like, what was exactly on your screen? If you didn't buy, why; and if you bought, what made you, you know, just do that? I'm asking you this in terms of your journey in purchase.

[01:50] Bhargav: In my case, why I did not go for buying the health insurance as of now was because I was diversifying my investments in other cases, and right now financially I'm not that stable to make the investment in terms of the insurance part. But if I had to do it, I would probably look at things like the network of hospitals that's been covered and the kind of health issues that the insurance can cover.

[02:21] Sagar: No, what I asked you was not that. What I asked you was like— okay, you have a health insurance?

Bhargav: No, not [yet].

Sagar: Okay, so now tell me something — have you ever, you know, tried applying for one, or did you, like, browse through any website or apps or anything?

Bhargav: I did come across — I think it was in some UPI apps and also in Policybazaar. Yeah, I did see the term coverage as well.

[02:54] Sagar: Okay, so tell me — when you are looking for the coverage, you know, they'll ask you for your details and everything, right? And the quote will be generated and everything. So I'm not asking you in terms of anything else; I'm asking you on the basis of your user journey of that particular website or app that you were looking at. So what, you know, stopped you from buying that particular insurance, or, you know, what would have made you purchase it?

[03:30] Bhargav: Maybe the clarity. Because 90% of the information that I have noticed that they provide in terms of insurance is just the financial package of what they are covering in it. They are not educating the investors in terms of what sort of diseases or what sort of network of hospitals you can claim it at. As you know, if insurance people are asking you to buy health insurance, then they will be having tie-ups or network hospitals in case the patient needs some treatment or operations. So I would say if they can probably explain this even more in depth, as well as what comes in and what does not come in the health insurance — if those clauses are explained — users like us would be able to make an informed decision.

[04:20] Sagar: Okay, so [apart from] the reason of you having your investments prioritized now and not purchasing the health insurance, these are the other reasons. Anything from the UI/UX standpoint?

[04:39] Bhargav: UI-wise, I feel if they can make it easier in terms of bringing all the information under one window. Because if you click on one health insurance, you are getting types of health insurance, but you are not getting the one that is quite customized to our own requirement. It's like — when you invest in mutual funds, you get those insights, right? How the performance has been in the last five years or anything. We should get some sort of, you know, insights like that in terms of what is the network of hospitals that they have, or—

Sagar: Well, they do provide the claim settlement ratio, right?

Bhargav: Claim settlement ratio, yeah, exactly. That, as well as the, you know, the conflicting issues — what comes under the plan and what does not come under the policy.

[05:31] Sagar: Okay. Okay. So, just tell me something. So there was one question that I asked you, like, you know, on that cataract surgery stuff.

Bhargav: Yes.

Sagar: Why did you answer that? How did you reach, you know, that answer?

Bhargav: You, I mean — if I remember, the answer was, I am not sure if that could be included in the insurance, right?

Sagar: Wait, wait, let me — just one second, wait.

[06:25] Sagar: Okay. You said you're not sure, for my question saying, you know, if a plan has a two-year waiting period for cataract, and any of your family members were covered and needed a cataract surgery after 10 months of you purchasing the plan — what happens? You said you were not sure. And why was that?

[06:52] Bhargav: In that case, I'm not aware of, like, under what conditions — since you said that it's under [a] waiting [period] — [whether we'd be] able to use the insurance to claim the cataract surgery. So I'm not sure whether that case is valid for us to go ahead and use the insurance, or it could be claimed under the insurance. So that is why I was of that opinion — that maybe I'm not sure whether it can be done or not.

[07:15] Sagar: Okay. So two more questions. So, like, when something has confused you, right, what did you do next?

Bhargav: [unclear] …or you would do research on that.

Sagar: Okay, yeah. So what are the ways that you do your own research to get more clarity?

Bhargav: Um… I reach out to some of the investors on YouTube, or in my community, where I actually reach out to my friends, friend circles who are already using health insurance. And as we know, in corporate we also have our own corporate health insurance, so we tend to reach out to their helpline, and also to understand how this health insurance has been structured and how, you know, we could claim, or what is covered in it.

[08:14] Sagar: Okay, but do you think your corporate insurance coverage is enough?

Bhargav: Never. I would never say that the corporate insurance would be enough, because, as you know, the [cap] will always be about close to 5 lakhs, and that would not suffice in case a major issue happens in terms of health. [unclear] Investment in a personal health insurance is always the safer choice.

[08:37] Sagar: Okay. How many of you are in your family?

Bhargav: [unclear] 4 people in the family. [unclear] One of them [unclear — "is insured"?] except mine.

Sagar: Okay. And, okay, what did your family ask that you couldn't answer with regards to health insurance?

[09:00] Bhargav: I mean, they did ask me as to why I have not opted for it. I mean, as I said, I have reasons for that. As of now, I have not yet reached the stability in terms of that, so that is why I have not gone through the plan of investing or getting myself insured.

Sagar: But don't you think it will be the right time to get yourself insured? Because as you age, the premium would go high.

Bhargav: [Good] point, of course. But right now, as I said, my break-even point is not that healthy to go ahead and invest. So once I'm probably more confident in terms of that, then I'll go ahead with it.

[09:36] Sagar: Okay, so one final question. So what is your, you know, coverage that you would be looking for, and what would be the premium that you would be willing to pay, in an ideal condition?

Bhargav: [I'd] pay up to 10, [unclear] the minimum premium. I feel that would be a reasonable method. [unclear] 10,000 rupees per month.

Sagar: Don't you think that's very high?

Bhargav: I mean, it depends on [unclear]. Like I said, I'm saying up to 10. That would be my max.

[10:24] Sagar: No, see, okay. Do you have any idea on what is the premium per year on health insurances?

Bhargav: I think it's about 500 [unclear].

Sagar: Right, so okay. So I'm asking you, what if you get health insurance, you know, for 25 lakhs of coverage — what do you think the premium would be?

Bhargav: [It would] be around 1,500, I guess.

Sagar: 1,500, yeah, that's safe to assume, right? So it will come to around 1,500 × 10 — 15,000 — around 17 to 18,000 rupees per annum, right? So this is what your traditional insurances offer, and they also have so many riders saying that they're not paying for consumables and everything.

[11:25] Sagar: Have you heard about this company called Acko, who is giving health insurance — 25 lakhs for 8,000, and where they cover 100 percent of your bills?

Bhargav: [unclear]

Sagar: Hmm, okay. Bro, your voice is breaking. Can you be a little louder and answer the question again?

Bhargav: I've heard of the Acko thing, but I've not heard in terms of the health insurance part of things. But yeah.

Sagar: So, thank you for your time. And this is all I have to ask you.

Bhargav: Thank you.

Transcript 5: Rekha (recorded interview)

Recording: Rekha_UI.mpeg (12 min 38 s) · Interviewer: Sagar

[00:01] Sagar: [unclear] …is recording. Okay, now first tell me what happened that, you know, made you— okay, first, before that, you tell me: do you have a personal health insurance apart from your office?

Rekha: No, I don't have a personal health insurance apart from the one provided by office.

Sagar: Any reason why?

Rekha: Honestly, I have been looking to get one, but it's been quite a difficult task. So I've just been leaving it, to be honest.

[00:35] Sagar: Right, right. Okay. So tell me — you said you were looking for it. What made you, you know, start looking for it?

Rekha: Because look, everyone needs to have a personal health insurance, right? That's a given. Everyone knows that. And you can't always rely on what office provides. And that's why — that was my intention of hunting for health insurance.

[01:02] Sagar: Okay. Okay. So now, let's say when you started looking, you would have, you know, seen some quotes, compared something, or, you know, viewed some information, right? So tell me, like, the exact moment that, you know, you stopped looking for that health insurance — why did you stop? Or if you, you know, confirmed that you want to buy it, or made the decision of purchasing it, what prompted you to do that? Both from your perspective and, like, from a user journey's perspective, right? Like anything with the UI/UX that you found off, or anything, even technical.

[01:43] Rekha: Um… so when I started looking for it online — the Policybazaar, Acko, whatever these platforms provide — there was so much online, too much to compare. I felt it was honestly overstimulating to look at so many policies. Plus, I didn't really know which one to pick — maybe lack of knowledge. I didn't know which one would cover everything that I need, etc. So it just didn't help. And another thing — hesitation was, even to look for something, you need to add your mobile number and email address, right? And you know the amount of calls you get the minute your number is out there.

Sagar: Understood.

Rekha: And then I switched to contacting an insurance agent instead, to have him do the work instead of me looking for it on my own. So he could do all the filtering and then bring to me a couple of options which I could pick from.

[02:52] Sagar: Yeah, okay. So the next question is — I asked, you know, let's say a plan has a two-year waiting period, right, a waiting period for cataract, and someone needed a cataract surgery after buying that policy, after 10 months — what happens? You said— one second… the plan pays in full.

Sagar: Wait, wait. Hello? Hello?

Rekha: Yeah, sorry, I couldn't hear you. Can you repeat?

Sagar: Yeah, so there was a question that I asked you where I said, you know, a plan has a 2-year waiting period for cataract, and someone needs a cataract surgery after 10 months of buying that particular policy — what happens? You said the plan would pay in full.

[04:09] Rekha: No, I don't think I said the plan would pay in full.

Sagar: Okay. [unclear]

Rekha: I don't think the claim would get accepted. The customer would pay in full is what I meant. Probably I read it wrong, I don't know.

Sagar: Okay. So you are aware about the waiting period clauses?

Rekha: Yeah, yeah, yeah.

Sagar: Understood, understood, understood. Uh…

[04:36] Sagar: Tell me something that confused you while looking for insurance, and what did you do next after being confused?

Rekha: There are so many aspects, right? Like, what are the diseases that it covers? What does it not cover? Plus there's something called a co-pay, various types of insurance that's available, and also, like, reimbursements, right? For example — I don't know if that's applicable only for life insurance or even for health, I don't know — let's say I've paid premium every month for a couple of years — I mean, I paid premium for many months that year and I didn't have to claim it. So will I in any way get a reimbursement, or rather, like, a repay sort of a thing? There's so many things. Sometimes it's confusing between life insurance and health insurance as well. So there are various aspects like that. All of those details, I always feel, are hidden in the fine print, and I don't really have that much time and knowledge to sit and go through everything, research each and every word. So I might as well go through an agent and ask him my queries directly, because he's well-versed in that area.

Sagar: So basically, transparency is something which is lacking.

Rekha: Okay.

[06:05] Sagar: So what did your family ask about health insurance that you couldn't answer?

Rekha: Uh… I don't recall off the top of my head, but I think we all have the same basic questions, right? How much is the premium? What is the coverage? What are the kinds of illness this premium covers? Is it a floater plan, like a family floater plan? Is there a top-up? Like basic stuff like that. I can't really think of anything specific off the top of my head right now.

[06:52] Sagar: Okay. Okay. So tell me something. So what do you think would be the appropriate sum insured that a person should look for today?

Rekha: I think a minimum of… ten.

Sagar: Okay, and what do you think the premium for that would be?

Rekha: Obviously it depends on the age and health-related [factors].

Sagar: Yeah, okay, let's say you are— okay, maybe for my age or something.

Rekha: Yeah. I mean, for a general population who's, you know, healthy enough, below, you know, 30 — anything between 25 to 30 [thousand?]. [unclear]

[07:44] Sagar: Okay. Okay. So on a usual day, a healthy individual, for a 25 lakh coverage, right, will have premiums, you know, somewhere around 16,000 rupees to 20,000 rupees, depending on the insurer [unclear]. So do you think that is too much or little?

Rekha: I think it's… I think it's [unclear] enough.

[08:30] Sagar: Okay. So have you heard about Acko, and what have you heard about them?

Rekha: It's a platform where you can buy policies for anything. It could be a car, it could be your health, anything.

Sagar: Right. So, unlike Policybazaar, Acko — they have their own, you know, policies. [unclear] And did you know that their policies are, you know, up to 50% cheaper than your conventional policies?

Rekha: No.

Sagar: When I checked for 25 lakhs there, the premium was 8,000 rupees per annum.

Rekha: Oh.

Sagar: This is not through any other— so basically, you know how you have these major agencies, right? Like Star Health Insurance…

Rekha: Yeah.

Sagar: So the difference is— so myself, I have a Niva Bupa. 25 lakhs, I pay 16,000 rupees.

Rekha: Okay.

Sagar: Acko, the same thing is at 8,000.

Rekha: Why is it cheaper?

Sagar: Because they don't have any middlemen. It's direct to consumer.

[09:53] Sagar: So, if given an option between Acko and conventional insurance like Star, Bajaj, you know, Niva Bupa, HDFC — what would you choose, and why?

Rekha: [If they're] offering [it] at a [lower] price compared to your conventional insurance policy, I would definitely check it out. I definitely would compare both to see if there's any major difference, right? In terms of — sometimes you opt for it thinking it's cheaper, and then you realize there are certain things that don't come along with it.

[10:38] Sagar: True. So, one thing to note about — so one thing to check is: let's say you have a medical bill of one lakh rupees and you claim it via any insurance, right? They would say — 70,000 or 75,000 rupees of that one lakh claim will be paid by the insurer, and the rest, the difference of whatever, 30 or 25 thousand, should be paid by you, because they are all consumables like syringes, gloves and all other things. And also they might have something like a room cap limit, which would determine other things also — [what's] paid and not. So some of these conventional insurers offer an add-on where you pay a thousand rupees extra or something and get even that covered. So effectively, your 1 lakh medical bill is covered in theory, but that's not actually true all the time, even though you have those add-ons. But Acko says that, you know, 100% of your medical bill is covered. What do you think about that?

[11:52] Rekha: Honestly, it sounds too good to be true, so I would look even a little more deeper into it, and then definitely, if it's all [unclear]—

Sagar: So "too good to be true" is the distinction between Acko and traditional insurance?

Rekha: You could say so. Maybe it's because I don't know much about it, and I have not come across anyone else who's used it, maybe. I would be a little skeptical, but given — if my needs are getting covered, then why not? I definitely would consider this. It's so much better. I'll be saving so much.

Sagar: So that's all the questions I had for you. Let me just stop the recording.

Transcript 6: Saurabh (recorded interview)

Recording: Saurabh_UI.mpeg (14 min 46 s) · Interviewer: Sagar

[00:00] Sagar: Okay, listen. So the first question for you is… okay, you have a health insurance?

Saurabh: Yes, I do.

Sagar: Is it a personal one? [unclear]

Sagar: Okay, cool. So tell me what made you, you know, start looking for health insurance?

[00:21] Saurabh: Because my company was giving it, and the coverage value was around five lakhs, of which I thought that I should probably take it in case of any adversities, or in case of any emergency it will be helpful for me to secure me and my spouse's health.

Sagar: No, but you don't have a personal one — you have a company one, right?

Saurabh: So that is— okay, you mean outside of the company?

Sagar: Yeah, yeah.

Saurabh: Outside of the company, I don't have.

Sagar: Okay, you don't have. Okay.

Saurabh: But do I need to say yes?

Sagar: No.

Saurabh: Bro, we can— I mean, you can record it again then.

Sagar: No, no, no, that's fine.

[00:55] Sagar: Okay, so also tell me, like, have you ever, you know, looked for an insurance outside the company, where you can buy it for yourself?

Saurabh: Currently no, because since I don't have any—

Sagar: Not currently. Let's say from the time you started working till date — have you, like, any time browsed around any of the apps or websites?

Saurabh: Yes, yes, I did, I did.

Sagar: So keeping that in mind, just tell me something. So what was that moment, right, on the screen, that stopped you — [that made you] not buy the insurance? Or, you know, what would have made you buy the insurance?

[01:40] Saurabh: So I will first answer what made me not buy the insurance: too many options. And too many options actually confuses you, and I had no right guidance on what to look for while purchasing any policy, what are the prerequisites, what should we be seeking in that policy, and what that policy should provide ideally. Second, why I selected it was — I chose the policy which benefited me more and which gave me a good medical health coverage in case of any accidents or any case of medical emergency.

[02:23] Sagar: Okay? So… any inputs from the UI/UX perspective?

Saurabh: Any input from the UI/UX perspective… What is UI/UX?

Sagar: I mean, we are on screen, right? Like all the information displayed, and your experience, or the user journey — like from viewing the policy to generating the quote — anything that has, you know, annoyed you, or anything that you feel could have been better.

[02:58] Saurabh: Yes, yes. I mean, they should actually provide a very concise description of each and every policy. And they should also tailor the policy as per the customer's needs. So if a customer doesn't want a particular— for example, if a customer opts for a particular policy, but in that particular policy there are some features which he doesn't wish to subscribe for, then I think these [providers] should also tailor some policies for the customers so that it suits their convenience. So this is what I personally feel, yes. So there should be a tailored policy for each and every customer.

[03:37] Sagar: Understood. And, uh, I had asked you a question regarding the, uh, cataract, right? Uh, one sec, just opening that up.

[04:09] Sagar: Okay, so the question was: if a plan has a two-year waiting period for cataract, and any of your family members would have been covered by it, right, and it has been 10 months after you bought the policy [and they need the surgery]… and you answered saying… uh… "You pay the full amount." Hello?

Saurabh: Yes, I [pay the] full amount, yes.

Sagar: So why did you say that?

Saurabh: Because I am the one who is earning as of now.

Sagar: No, [I meant] with regards to the policy. So you are aware of waiting [periods] and [unclear]?

[05:02] Saurabh: So that's why I said earlier there are some features in the policy which don't suit some customers, and a 10-month waiting period is quite long enough. So I don't want my parents or my relatives or my beloved ones to undergo or suffer something because of the delay. So I thought it's better that I pay from my own pocket and get this cataract operated rather than waiting for 10 months, because 10 months' time is quite a long time.

[05:31] Sagar: Okay, so that question was: you have bought the policy and there's a waiting period, but after buying the policy it's been 10 months. But usually the waiting periods, you know, are 3 years for some diseases, and for cataract there can be other clauses which I am also not sure about.

Saurabh: Yes, for some diseases I can understand, but it depends on the severity. But cataract is not as severe, and it should be operated as soon as possible. I mean, we should not wait for it for too long. Otherwise it might— I mean, I know it's not as severe as cancer or tumor or any other big diseases. But yes, I mean, again, it is connected with your eyes. And any kind of delay related to your eye or vision — I mean, it will affect your viewing capability, right? Your sight. It will impact your sight. So I thought I should not delay it further, and I should pay from my own pocket as of now.

Sagar: Okay.

Saurabh: Yes.

[06:37] Sagar: So when something confused you with regards to health insurance, what did you do next?

Saurabh: I… I thought [of reaching out to] policy experts, or I compared that policy with different— I mean, I compared policies from different websites. And the last resort was, I asked ChatGPT or Google Gemini to simplify the policy terms and conditions.

Sagar: Okay, when you said experts, who are they?

Saurabh: Policy experts — like, you know, in every website you have that AI assistant or the experts. If you don't understand any features, or if you don't understand any clauses in the policy, you can always reach out to those experts, and they will provide you a solution and they will explain it precisely.

[07:27] Sagar: Have you turned to YouTube? Influencers?

Saurabh: No. Because I don't trust YouTube influencers with policies.

Sagar: Okay.

Saurabh: Yes.

[07:36] Sagar: Okay. What did your family ask that you couldn't answer regarding the policies?

Saurabh: It was mostly regarding the payment. I mean, will the insurance company actually pay the money when it is required, or will they only pay a particular amount, stating that this particular thing is not covered in the policy? I mean, it's all related to policy clauses, that's it — which I couldn't explain to them.

Sagar: Understood.

Saurabh: Yes. So the clauses should be very precise.

[08:12] Sagar: Okay. Any reason that you don't have a personal policy?

Saurabh: Yep. As of now, because my company covers my medical— I mean, my company covers my dependents' health insurance. I mean, I have my company's insurance, so it…

Sagar: What is that [coverage]?

Saurabh: As of now, it provides me a coverage — it provides a 5 lakhs coverage. So as of now it's quite enough, but yes, I'm soon planning to purchase a personal policy as well.

Sagar: Okay.

Saurabh: Very soon.

[08:50] Sagar: So what do you think is the reasonable premium that you think you have to pay, and what do you think is a reasonable cover that one needs today?

Saurabh: Well, if I want to buy a personal policy — I mean, there is another option. Like, I can even opt for a premium one in my existing policy. So if I want to increase the cover, if I want to increase the policy—

Sagar: I mean, yeah, you can do that technically, yes. But it's still, you know, limited to the time that you're with your current company.

Saurabh: Yes, correct, correct.

Sagar: Right. So, like, there will be waiting periods, right, as you age and if you develop something. So it's always better to have a personal policy rather than just, you know, increasing the sum from the company itself, don't you think so?

[09:46] Saurabh: Yes, definitely. So I'm planning to buy a policy now. I will definitely— I'm planning to buy a policy. It's just that I just want someone who can explain to me all the clauses, terms and conditions very clearly, so that I don't face any problem in future. So if all these conditions are met, then I will…

Sagar: Okay. So what would be your ideal sum insured that you are looking for, and the premiums that you would be willing to pay?

Saurabh: I mean, ideally, 15 lakhs should be the sum insured. And the premium I can pay is like — it depends — if it's monthly 10,000 or 15,000, whatever the amount is, I'm ready to pay a 10,000 per month premium.

Sagar: 10,000 per month?

Saurabh: Yes.

Sagar: Per month?

Saurabh: Yes, correct.

Sagar: So that will be 1.2 lakh [per year].

Saurabh: Yeah, 1.2 lakhs a year. I mean, I'm just saying that I'm ready to pay that much. But if— I mean, if my current policy covers 15 lakhs, then I'm ready to pay more.

[10:46] Sagar: Okay. What do you think would be a premium for a 25 lakh rupee policy?

Saurabh: [For] 25 lakhs?

Sagar: Yeah.

Saurabh: So, I mean, uh… if you count it, then… let me check. Let me check. … It should be somewhere around 2 lakhs.

Sagar: [Are] you talking just for yourself, right?

Saurabh: Yeah, I mean, me and my dependents, right? My entire dependents — I mean, when it comes to — yeah, my dependents will also be covered in that, right?

Sagar: Okay. I mean, okay, I was asking you from a perspective of, you know, [just you].

Saurabh: I mean, you can calculate it that way also.

Sagar: But okay, so what if it is just for you and none of your dependents are covered?

Saurabh: Just for me, and if none of the dependents are covered, then I would not go for this much — I mean, not the larger sum. Then I would simply go for around 5 lakhs or 6 lakhs or 7 lakhs — below 10 lakhs I will go for.

Sagar: And what do you think the premiums would be?

Saurabh: Yeah, I mean, if it is 2 lakhs, then I will have to [pay] around 12,000 per month or like 15,000 per month. And if it's 5 lakhs… so accordingly I will have to pay somewhere around — I could say 41,000 if it's 5 lakhs.

[12:27] Sagar: Okay, I'll tell you something. So generally a 25 lakh rupee policy will cost you somewhere between 10 to 15,000 rupees — 12 to 15 thousand rupees per annum for a single person insured. Okay, so if you're going to take a group policy, you can, you know, assume how much it is.

Saurabh: [unclear] How come it's 15,000 per annum? But if it gave me 25 lakhs, it should be more, right?

Sagar: No, bro, it's just— okay, so I have my own policy, which is 25 lakhs, for which I pay 16,000 [and] some change.

Saurabh: But then that is not annual, right?

Sagar: It's annual.

Saurabh: And it's giving you 25 lakhs?

Sagar: 25 lakhs, yes.

Saurabh: Okay, because the thing is— I mean, is this recorded now?

Sagar: Yes.

Saurabh: Okay. Okay, so the thing is, I mean, I don't know exactly about this policy thing, but all I know is that the top-up depends on how much they are covering in that — what is the sum insured. Generally speaking, 10 to 15 thousand is right, but ideally, for 25 lakhs, I don't think it should be this much low. I don't know, maybe — I mean, I might be misinformed about it.

[14:08] Sagar: Okay. Have you heard about this company called Acko?

Saurabh: Acko… Acko, yes, I have heard — Acko insurance, yes.

Sagar: Yeah, have you heard about their health insurance?

Saurabh: Yes, I did.

Sagar: What's your take on that?

Saurabh: [unclear] I haven't [tried] that, because I have used Policybazaar.com. So I need to try Acko.

Sagar: [unclear] That's all the questions I have. I'm just stopping the recording.

Transcript 7: Abhinav (recorded interview)

Recording: Abhinav_UI.mpeg (18 min 25 s) · Interviewer: Sagar

Note: Abhinav's line was breaking up in several places, so this transcript has more [unclear] gaps than the others.

[00:01] Sagar: Okay. You have a personal health insurance?

Abhinav: Yes, I do.

Sagar: I mean not the office one — personal one.

Abhinav: Yeah, personal one.

Sagar: Okay, uh… tell me what made you look for it in the first place?

Abhinav: Come again?

Sagar: What made you buy a personal insurance in the first place? What made you, like, start looking for it?

[00:31] Abhinav: Okay. Initially, if you ask — I was thinking of buying my first insurance when I was working in my first company, and that was because everyone around me had a personal insurance.

Sagar: So you're talking about health, right?

Abhinav: Yes.

Sagar: Yeah, okay.

Abhinav: And [that] kind of created a FOMO in me. And that was also around the [time] of COVID. [unclear] [COVID] is one of the reasons — [it] created a FOMO in me, and then I had to look out for options, and then I eventually got [one]. [unclear]

[01:21] Sagar: So, no — tell me the moment when you were looking for the insurance that made you stop buying it, and later, you know, also made you, like, [say] "You know what, I'm going to buy this and make the payment." Like, what was on screen? What were, like, the decisions or the limitations or the factors that influenced all these decisions?

[01:55] Abhinav: So basically — just for my understanding — when I first looked at an insurance, why I didn't buy it, and later why did I go and buy it? Is it something like that?

Sagar: It's not something like that. Basically, let's say I'm browsing with an intent of buying an insurance. I go to Policybazaar or some website, and I look at an insurance. I say, "You know what, I'm not going to buy this." So that's the decision that you made of not buying it. And later you view some other insurance, or a different website or something, and you say, "You know what, I'm going to buy this."

Abhinav: So basically [you] wanted to know why I did not buy a certain insurance and why I did buy a [certain] insurance.

[02:43] Abhinav: Okay. So when I was first [looking], I actually surfed through a lot, and I didn't know how to buy it. I mean, I didn't know what [to] actually consider before buying. So I actually initially spent a lot of time looking out for a lot of options, and I did my research through YouTube and multiple other places, right? Asked my colleagues, asked my friends, relatives. [unclear] Eventually, when I started my research, I used the internet, and then I went through all of the options that are available in the market, and then I actually looked out for specific plans and all of that.

[03:32] Abhinav: So basically, when I first looked at the insurance plan — for me, specific things that were important were the coverage, right? I wanted the amount to be a little higher. And when I first initially looked at [one] insurance, the coverage was lesser, wherein I eventually just, uh… just dropped the plan of buying that particular insurance without even further pursuing it. And I also saw a lot of other [things], like some limitations on the rooms — [you'll] have to pay separately for the room, it is not covered under the package — and how the settlement process is, and how affordable the premiums are, and all of that.

Sagar: Okay.

Abhinav: And when I eventually took one, I found the coverage to be good, the premium is also reasonable, and the restoration benefit, and then the waiting period and all of that were pretty— you know, it was all pretty good. I had to take the one that would actually be feasible for me.

Sagar: Understood, understood.

[04:54] Sagar: Okay. So I had asked you a question, which is, like, regarding the cataract. So the question was: a plan has a 2-year waiting period for cataract. Someone in your family who has taken that insurance would require the cataract surgery after 10 months of buying that particular plan. What happens? And for that you said the plan pays in full. Why?

Abhinav: No — can you come again? Somebody has cataract [unclear]…

Sagar: [It has] a 2-year waiting period. Let's say a person has gotten a health insurance which has a 2-year waiting period for cataract, okay? And that person would require a cataract surgery 10 months after buying that plan. And who pays? You answered, "The plan would pay in full."

Abhinav: Okay. [Is] this the question you were asking before, right?

Sagar: Yes, it was in the questionnaire.

[06:25] Abhinav: Okay, I'll [answer] it. I… so basically, if I actually take, I mean, a plan that actually has a two-year waiting period, it wouldn't really fit for me. So for me, what I would need is a plan that can actually immediately pay for it — and see if there is an option to upgrade. I've seen a few plans where the basic ones will always have a waiting period; rather, if you pay more, you have a better plan.

Sagar: So you're talking about a waiver of the waiting period by [paying for a] rider?

Abhinav: Correct. See, that is really an [important] clause. And rather, I have to pay a little more to get a plan where I can just claim the money immediately. [I] think [that's] the most feasible [solution]. [unclear]

[07:28] Sagar: But you are aware of the waiting clauses, the waiting periods and everything else, right, in any particular insurance offering?

Abhinav: Yes. Okay.

[07:42] Sagar: So when something really confused you regarding health insurance, what did you do next?

Abhinav: In general?

Sagar: Yeah, generally — like, you're looking for health insurance and it's very confusing, right? There are so many things to, you know, look at. What would you do?

Abhinav: [unclear] I usually check on Google, or I surely use Gemini or ChatGPT to [help me] understand [the] terms. One is, try to understand how the exclusions work — like, what are all excluded in this plan, and all of that. And I [really] found it a hard time understanding when my claim would get rejected — what are the common reasons that, I mean, the insurer might reject my claim. So that was something that I had to spend a lot of time on.

[08:52] Sagar: Uh… what did your family ask that you couldn't answer?

Abhinav: [unclear] I was not questioned about [anything], to be really honest.

Sagar: Can you repeat it, because the voice is breaking?

Abhinav: I don't really get questions about all these because…

Sagar: No, I meant, like, when you were buying the insurance for yourself, or when your family members were buying it, you know, did they ask you any questions that you really didn't have an answer for — which you had to get clarity on from elsewhere?

[09:31] Abhinav: No. The first thing — the couple of questions they asked were not really about the insurance plan, but about myself: if I could really afford the premiums, how affordable they are, cash in the pocket. And second is if the insurer really has a somewhat trustable background, right? They are genuine, and they would, you know, release your claims and all of that. These were some of the [generic] questions that any family would have — [not] anything specific that I was not able to answer.

Sagar: Got it.

[10:10] Sagar: Okay, so now I have another question. So what would you say would be a [unclear]— okay, what's your sum insured that you have on your personal policy, apart from your office policy?

Abhinav: Sorry?

Sagar: What is your sum insured and the premium that you pay for your personal policy?

Abhinav: So… do you want the number?

Sagar: Yeah, yeah, number.

Abhinav: Okay. So… I… [unclear — checking] I don't have the—

Sagar: No, no, I meant approximate. I'm not asking you to the decimal; I'm just asking an approximate premium that you pay and the sum insured.

Abhinav: One sec. One sec. … Do you want the company name as well?

Sagar: No, company name is [not] relevant over here.

Abhinav: [unclear] So I pay an annual premium of 12 to 13,000 [?].

Sagar: Huh?

Abhinav: Okay, and I have… [unclear]

Sagar: You pay 12 to 14 thousand for a sum insured of 45 to 50 lakhs.

[11:53] Sagar: Okay, and do you think that's a reasonable thing?

Abhinav: The reason being, I wanted my coverage to be more. So I thought, rather than going to one crore or two crore at this age — where I'm just 26 — I thought it wouldn't make sense. But later [on], maybe I can do an add-on or [top-up]. 45 lakhs to [unclear] something, which I feel is [good enough] for the current phase of my life. So, yeah.

[12:32] Sagar: Okay, now — does your policy pay— let's say you have a claim of 1 lakh rupees. Does your policy pay one lakh rupees, or is there something that you will have to also pay out of your pocket? I'm not talking in terms of co-pay or something. I'm just talking in terms of [all your bills] being reimbursed or paid by the policy.

Abhinav: I still haven't claimed anything as such.

Sagar: Hmm. But are you aware that, you know, health insurance policies don't pay, like, 100% of your, what do you say, claim, unless you have an additional rider or an add-on in the policy?

Abhinav: I understand. So, [unclear] I'm able to understand that. See, I personally prefer a policy with no co-payment or no [room rent limit]. As I mentioned before. And as per my plan, what I've checked is that the only instance where I will have to be paying something out of pocket is when the treatment is excluded, or, uh, the co-payment clause.

[13:53] Sagar: No, no, no, I meant even if the treatment is included, right? For example, uh, you know, like if there's a surgery done or anything done, or even if it's just dengue or something and someone is admitted in the hospital, right? Obviously now your room is covered and everything is covered, but injections are being given, some medicines are being given — all these things fall into the category of consumables. So consumables are not covered by insurance — like gloves and everything. Right, so consumables are being paid out of your pocket, unless you have an add-on.

Abhinav: So my plan includes all of that, okay? It includes everything during hospitalization, inclusive of IV fluids, and treatment expenses, and even day-care treatment, and even outpatient treatment. All of these expenses are included in my OPD benefits.

Sagar: Understood, understood.

[15:04] Sagar: Okay, so have you heard about this company called Acko?

Abhinav: Acko, yeah.

Sagar: What do you understand about them?

Abhinav: So I [have] taken my vehicle insurance from them. [unclear]

Sagar: Okay, but do you know Acko has their own health insurance?

Abhinav: [I do.] [unclear]

Sagar: And what do you think about something like them? So when I checked, they were offering a 25 lakh health insurance cover at 8,000 rupees per annum, which is saying 100% bill payment, no deductions whatsoever. It's like a mobile-first platform kind of thing, where it's not like a traditional insurer — Star,

HDFC, Niva Bupa or any of those ones. What do you think about this kind of an offering, where everything is — they claim to be very transparent?

[16:15] Abhinav: [unclear] I mean, it's good. See, I'll be pretty honest with you. I don't have any issues with any of the vendors in the market, right? To me, what is more important is if somebody I trust, right — rather than me trying out something new — if I have someone who's already used a particular insurance vendor, and if they feel it's good, if they are [satisfied] with that, I am pretty open to go with them. I'm a little skeptical of [trying new] options with regard to these things, because I would always prefer a [referral] from [someone] who's used the product. So if someone has [recommended it to] me, I go [with that]. [unclear]

Sagar: [unclear]

Abhinav: I think — I mean, I'm not denying how good Acko is. They're pretty good too. I've heard a lot about them in the market. In fact, I even have a [vehicle?] insurance with them. Uh, but again — let me see. It all depends on the word of mouth and the good [reviews] I get from my peers. [unclear]

[17:24] Sagar: Do you think sometimes [an] insurance [offer] is also too good to be true?

Abhinav: Yeah, indeed, because… [unclear] see, [looking at] how the current scenario of the world is — especially during COVID and all that, I felt the importance of insurance. I mean, you never know how [big] a chunk of your money would go out just for your hospital expenses. I think [it's not just] a good-to-have thing — I feel we've all transitioned from having insurance as a good-to-have to [a] must-have. The world is transforming into such an environment that we all need insurance as a [unclear] for all of us. So, yeah, it is. It's valid.

Sagar: Understood. [unclear] …questions.

Transcript 8: Informal conversations at a Diwali gathering (field notes)

Interviewer: Sagar · Date: 8 November 2026 · Participants: ~10 adults (friends and acquaintances at a Diwali party)

Capture method: **Not recorded.** Conversations were face to face at a social event, where recording wasn't appropriate. Notes were written from memory afterwards.

How to read this document: *These are field notes, not transcripts. They record the themes heard across the conversations and approximate counts. They contain no verbatim quotes and don't attribute statements to named individuals. They are supporting evidence alongside the 7 recorded interviews (see *All_Interview_Transcripts*).*

1. Purpose and approach

- **Why:** To check whether the patterns from the 7 recorded interviews hold in a wider, informal setting.
- **Questions covered (same themes as the recorded interview guide):**
  1. Do you have personal health insurance beyond your employer's cover?
  2. Have you tried buying one online? What stopped you?
  3. What confuses you about health insurance?
  4. Where do you go when you have a doubt?
  5. Have you heard of Acko? Do you know Acko sells health insurance?
- **Format:** Short, unstructured conversations (one-to-one and small groups). Answers were noted from memory after the event.

2. Themes heard across the conversations

#	Theme	How often it came up	What people described
1	**Too many options**	Most conversations	Too many plans and insurers, with no clear way to choose between them
2	**Lack of transparency**	Most conversations	Unclear what is actually covered and what will be paid at claim time
3	**Trust gap when an offer looks transparent**	Several	When an insurer does claim full transparency or very low prices, it feels "too good to be true" and isn't believed
4	**Spam calls after sharing a phone number**	Very common, often the strongest complaint	Entering a mobile number on Policybazaar or insurer sites leads to repeated daily calls until the numbers are blocked
5	**Price and payment plans**	Some	Some policies feel expensive; some insurers lack flexible payment plans
6	**Confusing insurer websites, no comparison**	Several	Insurer sites (HDFC, Axis, Kotak given as examples) are confusing and only show their own plans, with no comparison against other insurers

#	Theme	How often it came up	What people described
7	**Comparison happens elsewhere**	Several	The only places people felt they could compare were Policybazaar or AI tools (ChatGPT, Gemini)
8	**Where doubts get resolved**	Common	YouTube, friends, ChatGPT and Google

3. Acko awareness (approximate, out of ~10)

Signal	Approximate share	Approx. people
Aware of the Acko brand	Most	Majority of the group
Recalled seeing an Acko ad	~20%	~2
Use Acko for motor insurance	~40%	~4
Aware Acko sells health insurance	A few	Small minority
Bought Acko health insurance	~10%	1 (purchased ~7 days before the conversation; left before follow-up questions)

Counts come from recollection, not a structured tally. Treat them as directional, not measured.

4. Notable individual observation

The one Acko health insurance buyer. One person had bought Acko health insurance about 7 days before the party. They confirmed the purchase but left before any follow-up questions about their buying journey. This person is the only first-hand Acko health customer met during the research and is the priority for a recorded follow-up interview.

5. Synthesis of the Diwali conversations

What stops people from buying:

- **Decision paralysis:** too many plans, and no clear way to compare them.
- **Distrust:** coverage isn't transparent, and when an insurer does claim transparency (or a very low price), people suspect a catch.
- **Fear of spam:** sharing a mobile number to see a quote leads to daily sales calls, so people avoid starting the journey at all.
- **Cost:** some find premiums high or payment options rigid.

Gaps in the current market:

- Insurer websites only show their own plans, with no comparison against other insurers.
- Comparison has moved to Policybazaar (which brings the spam) or to AI tools like ChatGPT and Gemini.
- Doubts are cleared through YouTube, friends and Google rather than the insurer.

What this means for Acko:

- Brand awareness is high, and motor insurance is the entry point (~40% are motor customers).
- Awareness of the health product is low, so the motor base is an untapped cross-sell audience.
- A spam-free, comparable, transparent quote journey would address the three strongest complaints directly.

Section 6 of the original notes (a comparison with the recorded interviews) and section 7 (limitations) appear in the research summary, so they are not repeated here.

Research summary: all 8 transcripts

Combined from the three summaries written during the research. Where two summaries said the same thing, the more detailed version is kept.

1. Research overview

17 people were spoken to about buying personal health insurance, through two methods.

Source	People	How captured	Evidence status
Recorded user interviews (phone/voice)	7: Adhira, Aditya, Manasa, Bhargav, Rekha, Saurabh, Abhinav	Audio recordings (1.5 to 18.5 min), machine-transcribed; preceded by a survey of 25	Primary: full transcripts available
Informal conversations at a Diwali gathering (8 Nov 2026)	~10	Not recorded; field notes written from memory	Supporting: aggregate field notes

Headline finding: The main barrier isn't price. People can't tell what a policy will actually pay, they're overwhelmed by choice, and they don't trust the sellers. Acko is well known, but mostly as a motor insurer. Its health offer is barely known, and when people hear it, it sounds too good to be true.

2. Themes across both groups

Theme	Recorded interviews (7)	Diwali conversations (~10)	Strength
Coverage and claim payout unclear; fine print	6 of 7 (Manasa got 40–50% of a family bill paid)	Most	Very strong
Too many options, no guidance	Rekha, Saurabh, Adhira, Abhinav	Most	Very strong
Spam calls after sharing a phone number	Rekha	Very common, often the strongest complaint	Strong
"Too good to be true" distrust	Manasa, Rekha	Several	Strong
No cross-insurer comparison on insurer sites	Bhargav (wants one-window insights)	Several (HDFC, Axis, Kotak named as examples)	Strong
Help sought on YouTube, from friends, AI tools, Google	Bhargav, Abhinav, Saurabh	Common	Strong
Low insurance literacy	5 of 7 couldn't answer the waiting-period question at first	Not specifically tested	Moderate
Price and payment plans	Bhargav (affordability), Manasa (high premium due to PED)	Some	Moderate
Pre-existing condition penalties	Manasa	Not raised	Single but deep signal
Employer cover feels "enough"	Aditya, Saurabh, Manasa	Not raised	Moderate

3. Interview participants at a glance

Five of the seven have looked for a personal policy and stalled. One isn't looking, and one has already bought elsewhere.

Participant	Personal policy?	Stage	Main blocker or driver	Cataract waiting-period question	Acko awareness
Adhira	No (employer only)	Researching	Can't decide between offers; unhappy with coverage; unclear sub-clauses	Not aware of waiting periods	Heard the name; doesn't know what they do
Aditya	No (employer only)	Not looking	Thinks employer cover is enough; can't recall his own sum insured	Not aware	Heard the name; unaware of health product
Manasa	No (employer only)	Looked, dropped off	Thyroid led to loading, waiting periods, high premium; family claim paid only 40–50% of the bill	Correct	Seen many ads; never explored
Bhargav	No	Browsed (UPI apps, Policybazaar)	Affordability and other investments first; info is only financial, not diseases or network hospitals	Unsure	Knows Acko; unaware of health product
Rekha	No (employer only)	Looked, then switched to an agent	Too many options; spam calls after sharing her number; fine print	Correct after clarifying	Sees it as an insurance platform; unaware of price gap
Saurabh	No (employer, ₹5L)	Browsed; plans to buy "very soon"	Too many options, no guidance, wants clauses explained	Misread as a personal choice	Aware of Acko health; used Policybazaar instead
Abhinav	Yes (~₹45–50L, ~₹12–14K/yr)	Bought (driven by FOMO and COVID)	Dropped plans with low cover or room-rent caps	Wants a waiting-period waiver	Has Acko vehicle insurance; buys only via referrals

4. Pain points and reasons for not buying (recorded interviews)

1. **Opaque coverage and fine print (6 of 7).** Exclusions, sub-limits, room-rent caps and consumables are buried in about 50 pages of terms. Manasa expected 80% of a family hospital bill to be covered and got 40–50%.

2. **Low insurance literacy (5 of 7).** Only Manasa answered the cataract waiting-period question correctly at first. Rekha got it right after clarifying. Co-pay, consumables, and life vs. health insurance are confused. Premium expectations range from ₹500 to ₹1.2L a year.

3. **Choice overload and no guidance (4 of 7).** Rekha, Saurabh, Adhira and Abhinav all said there are too many plans and no clear criteria to choose by.

4. **Spam calls after lead capture (Rekha).** A phone number is required before seeing quotes, and it brings a flood of sales calls.

5. **Pre-existing conditions penalised (Manasa).** Loading, waiting periods and high premiums were not explained before payment.

6. **"Employer cover is enough" inertia (Aditya, Saurabh, Manasa).** Bhargav disagreed: a ₹5L corporate cap is never enough.

7. **Affordability and competing priorities (Bhargav).** Investments come first until his finances feel stable.

Where people go when confused: Policybazaar (Manasa, Saurabh, Rekha), ChatGPT, Gemini and Google (Saurabh, Abhinav), YouTube (Bhargav, Abhinav; Saurabh distrusts it), friends and colleagues (Bhargav, Abhinav), an agent (Rekha), the corporate insurer's helpline (Bhargav).

5. What users want

Need	What it looks like	Raised by
Plain-language covered / not-covered summary	One-page category view instead of 50 pages of T&C; common claim-rejection reasons	Manasa, Rekha, Saurabh, Bhargav, Abhinav
Bill-level payout clarity	"Of a ₹1L bill, you pay X" (consumables, room rent, co-pay shown upfront)	Manasa, Saurabh, Abhinav
Upfront pre-existing disease pricing	Loading and waiting period shown before payment	Manasa
Comparison and insights	One window with network hospitals, diseases covered and claim settlement, like mutual-fund insights	Bhargav, Saurabh
Tailored or modular plans	Drop unwanted features; pay to waive waiting periods	Saurabh, Abhinav
Browse without sharing a phone number	Quotes without lead capture	Rekha
Trusted explainer	Agent, expert or AI that answers clause questions	Rekha, Saurabh, Abhinav
Social proof	Referrals and reviews from people who have claimed	Abhinav, Rekha

Target cover: ₹10–25L is the most common target (Manasa ₹15–20L at ~₹15–20K/yr; Rekha at least ₹10L; Saurabh ₹15L).

6. Acko awareness and perception

Signal	Recorded interviews (7)	Diwali conversations (~10)
Heard of Acko	7 of 7	Most
Recalled Acko advertising	Manasa	~20% (~2)
Acko motor customer	Abhinav	~40% (~4)
Knew Acko sells health insurance	Saurabh, Abhinav	A few
Bought Acko health insurance	0	1 (bought ~7 days earlier)

People know the brand, but very few know about or buy the health product. The motor customer base is the obvious bridge.

From the recorded interviews:

- When told about the ₹25L-for-₹8K, 100%-of-bill offer, Manasa and Rekha both said it sounded "too good to be true". Both would still consider it after checking.
- Trust comes from people, not ads. Abhinav buys only on peer referral, and Rekha knew no one who had used Acko.
- Price is a strong pull once understood. Rekha said "I'll be saving so much."

7. Opportunities for Acko

1. **Payout simulator:** enter a sample bill and see exactly what's paid. This answers the top question across all 17 people.

2. **No-spam promise:** quotes without a phone number, which turns the most common complaint into a reason to choose Acko.

3. **Proof against "too good to be true":** publish claim amounts settled, not just the claim settlement ratio, and show real claimant stories.

4. **Upfront pre-existing condition pricing** before payment.

5. **Cross-sell to motor customers:** about 40% of the Diwali group already trust Acko for motor insurance.

6. **Built-in AI clause explainer:** users already turn to ChatGPT and YouTube for this.

8. Limitations

- Small convenience sample drawn from the researcher's network: salaried professionals in their 20s and 30s.
- Leading prompts in 4 recorded interviews (the Acko offer was described before asking for a reaction).
- Some recordings had poor audio ([unclear] gaps), especially Abhinav's.
- Diwali conversations weren't recorded, so there are no verbatim quotes; notes and figures depend on recall and are approximate.
- The one Acko health buyer met at the Diwali gathering hasn't yet been interviewed in depth. A recorded follow-up is the next step.